

HOW TO OPEN An Adult Care Facility or Assisted Living Residence in New York

A step-by-step startup guide — the license stack, staffing, and how you get paid

Regulatory basis: SSL Article 7 · 18 NYCRR Parts 485–490 (adult care facilities) · PHL Article 46-B and 10 NYCRR Part 1001 (assisted living residences) · 18 NYCRR Part 494 (Assisted Living Program)

Oversight: New York State Department of Health (DOH), Bureau of Licensure and Certification, with Local Departments of Social Services

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The Big Picture

New York does not have a single “assisted living license.” It has a stack of them, and understanding the stack is the first and most important step. The base license is an adult care facility (ACF): a residence that provides room, board, housekeeping, supervision, and — in most types — personal care to five or more adults who cannot live fully independently but do not need the continuous medical or nursing care of a hospital or nursing home. The New York State Department of Health licenses and surveys these facilities.

On top of the ACF, an operator may add certification as an assisted living residence (ALR) — the modern 2004 model that adds 24-hour on-site monitoring, an individualized service plan, and daily food service, with Enhanced (aging-in-place) and Special Needs (dementia) sub-types. And an operator may add the Assisted Living Program (ALP), a Medicaid service overlay. A single building can hold one license, two layered together, or all three, and which combination you hold determines who can live there and who pays.

There is also a smaller entry point: a family-type home for adults, which serves up to four adults and is licensed at the local level. This guide focuses on the adult home / enriched housing base plus ALR certification — the core “assisted living” path — and is honest about the part everyone asks about: how you actually get paid. It is general educational information, not legal advice; confirm every step, figure, and contact with DOH and your Local Department of Social Services before you rely on it.

The License Stack — What Each One Means

Layer	What it is	Who pays
Adult Care Facility (ACF)	Base license — adult home (Part 487), enriched housing (Part 488), or residence for adults (Part 490). Room, board, supervision, personal care.	Private pay, or SSI + State Supplement for room & board
Assisted Living Residence (ALR)	Certification on top of an adult home/EHP (PHL 46-B; 10 NYCRR 1001), with Enhanced (EALR) and Special Needs (SNALR) sub-types.	ALR-only is essentially private pay
Assisted Living Program (ALP)	Medicaid service overlay (Part 494) on an adult home/EHP; capped statewide and released through periodic competitive solicitations.	Medicaid for services + SSI + State Supplement for room & board
Family-Type Home (FTHA)	Up to 4 adults; licensed at the Local DSS level (Part 489). The small-scale entry point.	Private pay or SSI + State Supplement

The Path at a Glance

1. Decide your model — which base license (adult home or enriched housing), whether to add ALR certification, and whether to pursue the ALP — and set up the business (entity, EIN, workers' compensation and disability coverage).
2. Have a pre-application conference with DOH; New York recommends it before you apply.
3. Secure a location and confirm local zoning; think ahead to DOH architectural review.
4. Submit the Adult Care Facility Common Application to the DOH Bureau of Licensure and Certification, which is judged on public need, financial feasibility, and character and competence.
5. Complete DOH architectural / plan review and local building, fire, and sanitation approvals.
6. Receive Part I (establishment) approval, then submit Part II (program information) within the required window.
7. Add ALR certification if desired (you must hold or obtain the adult home / enriched housing base first).
8. For the ALP, respond to a DOH Request for Applications when beds are made available — ALP capacity is capped and released competitively.
9. Hire and screen staff, adopt your policies, and get survey-ready.
10. Receive your operating certificate (both Part I and Part II must be approved before admitting residents), then connect residents to SSI + the State Supplement and, where applicable, ALP Medicaid — and open.

Part 1 — Choose Your Model and Set Up the Business

Start with the license stack. The most common base licenses are the adult home (congregate rooms, personal care, at least five residents) and the enriched housing program (apartment-style units, community-integrated, at least one hot congregate meal a day). Enriched housing is often the substrate operators pair with the ALP because its apartment configuration fits home-and-community-based-services rules. Decide whether you will add ALR certification for the modern assisted living model, and whether you will pursue the ALP for Medicaid-funded services. If you are starting small, the family-type home for adults (up to four adults, licensed by your Local DSS) is a simpler entry point.

Then set up the business like any care company: form your legal entity, get an EIN, and put workers' compensation and disability insurance in place — New York requires proof of coverage. Keep your entity information consistent everywhere, because DOH reviews the legal structure closely and, if you pursue the ALP, the ownership of both the facility and the home care component is examined. Budget realistically: a new adult care facility can take 18 months to two years to move through construction, local permits, and DOH licensing.

Part 2 — The DOH Application: Need, Feasibility, Character

Adult care facilities are licensed by the DOH Bureau of Licensure and Certification through the Adult Care Facility Common Application, working with the appropriate DOH regional office. New York recommends a pre-application conference — take it; it can save months. Like other New York provider approvals, the application is judged on three things: public need, financial feasibility, and character and competence.

Financial feasibility means showing you can fund the project and operate it — typically with a certified public accountant confirming your resources and estimating operating costs for the first and third years. Character and competence is a detailed questionnaire completed by all owners covering education, employment, licenses, health-facility affiliations in and outside New York, and any legal actions; out-of-state affiliations trigger an out-of-state compliance review. Assemble these carefully and completely, because incomplete or weak submissions are the most common cause of delay.

Part 3 — Architectural Review, Building, Fire and Sanitation

If your project involves construction or renovation, DOH conducts an architectural / plan review against the environmental and structural standards for adult care facilities, and you must attest to building to the New York State building codes. You may request approval to commence early construction at your own risk, but final approval is required before residents move in. In parallel, you obtain the local building, fire-safety, and sanitation approvals your municipality requires.

Design to the standards from the start. The building's exits, fire-safety systems, unit configuration, and accessibility affect your capacity and the residents you can serve, and enriched housing has its own capacity rules tied to the building. Involve DOH and your architect early; plan review is where long timelines get longer, and a building that looks ready can still need changes to meet code for a licensed residence.

Part 4 — Part I, Part II and the Operating Certificate

The application runs in two parts. Part I is establishment — DOH's decision that you (the operator) and the project should be approved, based on need, feasibility, and character and competence. After Part I approval you submit Part II (Schedule 6, program information) within the required window, describing how the facility will actually operate. Both Part I and Part II must be approved before you admit any residents, and DOH issues — or revises — your operating certificate when the project is approved.

Your operating certificate states your facility type, capacity, and any certifications. Post it, operate within it, and seek DOH approval before any change in operator, capacity, category, location, or certification that requires it. Operating certificates are renewed periodically; keep yours current along with your inspection results and any plans of correction.

Part 5 — Adding ALR Certification and the ALP

To certify as an assisted living residence, you must first hold — or obtain at the same time — an adult home or enriched housing program operating certificate; the ALR is a certification layered on that base, with a character-and-competence review and the consumer-information and residency-agreement requirements of 10 NYCRR Part 1001. From there you can seek Enhanced (EALR) certification to admit and retain residents who need more physical assistance so they can age in place, or Special Needs (SNALR) certification for dementia care, each with additional requirements.

The Assisted Living Program is different: it is a Medicaid program with a statewide cap of 4,200 residents, and new ALP beds are released only through periodic competitive Requests for Applications announced by DOH. You cannot simply apply for ALP capacity whenever you wish — you respond to a solicitation using the Common Application. If Medicaid-funded services are central to your plan, watch DOH's announcements and be ready to apply when beds are offered.

Part 6 — Staffing, Screening and Training

Staff the facility for an organized 24-hour program of supervision and services — an ALR provides 24-hour on-site monitoring — with enough qualified people for your census and residents' assessed needs. Screen staff before they serve residents, including the criminal history and abuse/registry checks required for adult care facility staff, and complete staff health and tuberculosis screening. Remember New York's medication distinction: adult home staff assist residents with the self-administration of medications rather than administering them as a nurse would.

If your facility is subject to the Justice Center for the Protection of People with Special Needs, you also carry Justice Center obligations — checking prospective staff against the applicable exclusion systems and training staff to report reportable incidents to the Vulnerable Persons' Central Register. Train all staff on resident rights, incident recognition and reporting, infection control, and emergency procedures, and add the ALR and dementia-specific training the rules require. Document every screening, training, and competency.

Part 7 — Policies, Records and Getting Survey-Ready

You must operate from written policies and procedures covering resident care and facility operation, kept current and available to DOH. That is exactly what the PolicyReady Adult Care Facility & Assisted

Living Residence Policy & Procedure Manual provides, mapped to 18 NYCRR Parts 485–490 and 10 NYCRR Part 1001. Adopt it, tailor it to your facility, train staff on it, and review it at least annually.

Set up resident records before your first admission: the admission or residency agreement, the assessment and individualized service plan, medication information and records of the assistance provided, case-management and progress notes, incident records, and the signed statement of resident rights (and, for an ALR, the Resident's Bill of Rights). Post the resident rights, the grievance procedure with Ombudsman contact information, and your emergency procedures. Being survey-ready on day one is the difference between a clean first inspection and an early plan of correction.

Part 8 — Getting Paid: SSI, the State Supplement and the ALP

Here is the honest money picture, and it is different from most states. New York separates the two halves of the bill. Room and board is paid from the resident's own income; for low-income residents that means SSI plus the New York State Supplement (SSP), administered by the Office of Temporary and Disability Assistance and set by “congregate care level.” Care and services, for those who qualify, are paid by Medicaid — but only through the Assisted Living Program, and Medicaid never pays room and board.

The catch on the Medicaid side is real: the ALP is capped at 4,200 residents statewide, is not an entitlement, and forms a waitlist when full; an applicant must be assessed (through the Uniform Assessment System) as needing nursing-home level of care but not continuous nursing, and must be able to evacuate with assistance. An assisted living residence without the ALP is essentially private pay. Plan your payer mix around this.

Funding at a glance (2026 figures — verify current amounts with your Local DSS, SSA, and NY Medicaid; rates change):

Source	What it pays for	Amount	Key conditions
SSI + NY State Supplement — Congregate Level 3	Room & board (enhanced residential / assisted living)	Up to ~\$1,688 / month (individual, 2026)	Resident contributes income minus a \$262/month personal needs allowance; assets under \$2,000
SSI + State Supplement — other levels	Room & board in an adult home (varies by level and region)	SSI FBR \$994 individual / \$1,491 couple, plus SSP by living arrangement	SSP living alone \$87, living with others \$23 (2026); varies by congregate level
Assisted Living Program (ALP) — Medicaid	Personal care / home care services and case management (not room & board)	Medicaid rate paid to the facility for services	Capped at 4,200 statewide; UAS-NY assessment; nursing-home level of care; Local DSS approval; not an entitlement
Private pay	ALR services and the balance of costs	Market rate (varies widely by region)	ALR-only facilities are essentially private pay

ALP financial eligibility in 2026 is generally income up to about \$1,836/month for an individual (\$2,489 for a couple) with assets under \$2,000 — confirm current limits and the SSI/SSP congregate-care rates with your Local DSS and SSA. Apply for Medicaid and ALP services through the Local Department of Social Services.

Part 9 — After You Open: Surveys, Reporting and Staying Compliant

DOH surveys adult care facilities and investigates complaints, and the Adult Care Facility Centralized Complaint Intake line receives concerns. Treat continuous readiness as daily operation: keep your operating certificate, policies, resident and personnel records, drills, and inspections current. When DOH cites a deficiency, submit and implement a plan of correction within the required timeframe and sustain it.

Know your reporting duties. Reportable incidents go to DOH within required timeframes, and if your facility is subject to the Justice Center, to the Vulnerable Persons' Central Register. Involuntary discharge requires a 30-day written notice and, if contested, a special proceeding under Social Services Law Section 461-h. Build a compliance calendar of recurring obligations and run an annual self-assessment against the rules and your manual.

Part 10 — Finding and Admitting Residents

Residents come through defined channels: your Local DSS (which handles Medicaid, ALP approval, and SSI/State-Supplement matters and is aware of people needing placement), hospital discharge planners and social workers, nursing facilities discharging to a lower level of care, physicians, the Long-Term Care Ombudsman and area agencies on aging, and families searching directly — the NYSDOH Adult Care Facility directory lists licensed facilities. The facilities that stay full are easy to reach, respond the same day, and are honest about who they can serve.

Admit only residents whose assessed needs you may lawfully meet within your license — remember that adult care facilities may not admit or retain people who need continuous nursing or medical care — complete the assessment and a written residency agreement, provide the statement of resident rights, and start the resident's records and service plan from day one. (PolicyReady also offers a referral system built to fill your beds through these channels.)

What It Costs to Start

Exact startup cost depends heavily on your model, your building, and your region — New York City and downstate are far costlier than upstate — so treat the categories below as a planning checklist and get real quotes. The building and the long approval timeline are the two biggest financial realities.

- **Business setup.** Legal entity, EIN, business banking, and the workers' compensation and disability insurance New York requires.
- **Building to code.** Purchase or lease, plus construction or renovation to meet the environmental and structural standards and fire code for your type and capacity — usually the largest and most variable cost.
- **Professional and application costs.** Architect and, for financial feasibility, a certified public accountant; legal and consulting help with the Common Application; local permit fees.
- **Carrying costs during approval.** Because approval can take 18 months to two years, budget to carry the property and project through the review period before any revenue.
- **Staffing and training.** Recruiting, background and registry checks, health screening, and training — including ALR, dementia, and Justice Center training where applicable.
- **Operations and furnishings.** Furnishings, kitchen and food service, a call/monitoring system, supplies, and initial stock.
- **Insurance and reserves.** Liability and other coverage, plus working-capital reserves for the months before SSI/State-Supplement and any ALP payments stabilize cash flow.

Serving Residents with Dementia (SNALR)

If memory care is part of your plan, decide early whether to certify as a Special Needs Assisted Living Residence (SNALR). New York treats dementia care as a distinct certification on top of the ALR, with additional program, staffing, training, and environmental requirements and specific consumer disclosures about what makes the care 'special.' An Enhanced ALR (EALR) is the related certification that lets you admit and retain residents who need more physical assistance so they can age in place.

These certifications shape your building (for example, secure, supportive environments for residents at risk of wandering), your staffing and training, and the residency agreement addenda you must use. They also affect who you can serve and how you market — describing a residence as memory care carries real obligations — so build the certification in from the start rather than retrofitting it, and confirm the current SNALR and EALR requirements with DOH.

Common Mistakes to Avoid

- **Misreading the license stack.** ACF, ALR, and ALP are different things with different payers — decide which you need before you design the building or the budget.
- **Assuming you can 'apply for the ALP' anytime.** ALP beds are capped statewide and released only through periodic competitive solicitations — watch DOH announcements.
- **Skipping the pre-application conference.** New York recommends it, and it can prevent costly missteps in a long approval process.
- **Underestimating the timeline.** New construction can take 18 months to two years through permits and DOH licensing — plan cash flow accordingly.

- **Weak character-and-competence or feasibility submissions.** Incomplete owner disclosures or unverified finances are common causes of delay or denial.
- **Admitting residents before full approval.** Both Part I and Part II must be approved first, and you may not admit people needing continuous nursing or medical care.
- **Treating medications like a nursing home.** An adult home assists with self-administration; it does not administer medications as a nurse would unless authorized under its certification.
- **Assuming Medicaid covers room and board.** It does not — room and board comes from SSI + the State Supplement or private pay; Medicaid (ALP) pays only for services.

Your First Steps

Because New York's timeline is long, think in phases rather than 90 days. First, decide your license stack and form your entity with workers' compensation and disability coverage, then request a pre-application conference with DOH and confirm zoning at your intended site.

Next, assemble the Adult Care Facility Common Application — the project description, the financial-feasibility package with your CPA, and the character-and-competence questionnaires for all owners — and begin architectural review and local approvals. Pursue Part I establishment, then Part II, and add ALR certification if desired; watch for an ALP solicitation if Medicaid services are part of your plan.

Finally, in the run-up to opening, hire and screen staff, complete required training including any Justice Center obligations, adopt and tailor your policy manual, stand up your resident-records and medication systems, and do a mock survey against your manual. Admit your first residents — with complete records — only after both parts of your approval are in hand.

Local Call List — New York City

Verified starting contacts for opening in New York City. In NYC many services route through 311; confirm the right office and current number when you call.

Agency	Why you call them	Phone
NYC Human Resources Administration (DSS)	Medicaid, ALP approval, SSI/State-Supplement and benefits matters	212-835-8300
NYC Department of Buildings	Building permits, plan questions, inspections	212-566-5000
FDNY / fire safety (via 311)	Fire-safety review, inspections, permits	311

Licensing itself is handled by DOH (see the statewide table), working through its New York City regional office. For fire and many city services, 311 routes you to the right unit.

Local Call List — Buffalo (Erie County)

Verified starting contacts for opening in the Buffalo area. Confirm the right division and current number when you call.

Agency	Why you call them	Phone
Erie County Dept. of Social Services	Medicaid, ALP approval, SSI/State-Supplement; FTHA questions	716-858-8000
Erie County Department of Health	Sanitation / environmental health; local health requirements	716-858-7690
City of Buffalo Inspections Department	Building permits and inspections	716-851-4937

For fire-safety sign-off, contact the fire marshal / fire department for your municipality. Licensing is handled by DOH through its regional office serving western New York (see the statewide table).

Statewide Contacts

Office	Purpose	Phone
DOH Bureau of Certification & Finance (Adult Care / Assisted Living)	Licensing and certification (875 Central Ave, Albany)	518-408-1624
DOH Adult Care / Assisted Living application intake	Application and certificate-of-need questions	518-408-5287
Adult Care Facility Centralized Complaint Intake	Complaints about an adult care facility / adult home	866-893-6772
NY State Long-Term Care Ombudsman	Residents' rights; regional ombudsmen	855-582-6769
Justice Center — Vulnerable Persons' Central Register	Report abuse/neglect (reportable incidents), 24/7	855-373-2122
Justice Center — report a death	Reporting a resident death where required	855-373-2124
Local Department of Social Services	Medicaid, ALP approval, SSI/State Supplement, FTHA	Your county / NYC

Quick Glossary

- **ACF.** Adult care facility — the base license (adult home, enriched housing, or residence for adults).
- **Adult home / Enriched housing.** The two common ACF types — congregate (Part 487) and apartment-style (Part 488).
- **ALR / EALR / SNALR.** Assisted living residence certification and its Enhanced (aging-in-place) and Special Needs (dementia) sub-types.
- **ALP.** Assisted Living Program — the Medicaid service overlay; capped statewide and released by competitive solicitation.
- **FTHA.** Family-type home for adults — up to four adults, licensed by the Local DSS.
- **SSI + SSP.** The resident's federal SSI plus the New York State Supplement that pays room and board, set by congregate care level.
- **UAS-NY.** The Uniform Assessment System used to determine ALP/Medicaid eligibility and need.
- **Operating certificate.** The DOH license stating your facility type, capacity, and certifications.

Core Rules to Keep Close

- SSL Article 7 and 18 NYCRR Parts 485–490 — adult care facilities (adult homes 487, enriched housing 488, residences for adults 490).
- PHL Article 46-B and 10 NYCRR Part 1001 — assisted living residences (including PHL 4660, the Resident's Bill of Rights).
- 18 NYCRR Part 494 and SSL 461-l — the Assisted Living Program.
- SSL 461-h — involuntary discharge protections; SSL 461-m — reportable incidents and the Justice Center.
- 18 NYCRR 487.7 — resident services, including assistance with self-administration of medications.

This guide is general educational information for prospective New York adult care facility and assisted living operators and is not legal advice. Rules, fees, figures, and contacts change — verify each with the New York State Department of Health, your Local Department of Social Services, and NY Medicaid before relying on it. Pair this guide with a current policy and procedure manual and a complete forms set, and confirm your specific obligations before you open.